Psychiatric Mental Health Nursing Department

Personality Disorders

Objectives:

At the end of this lecture students will be able to:

- Define personality disorders to clarify what constitutes these mental health conditions.
- Examine the incidence and prevalence of personality disorders to understand their impact on the population.
- Identify the risk factors associated with personality disorders to highlight potential causes and vulnerabilities.
- Classify the different types of personality disorders to categorize them based on their characteristics.
- Explore the treatment modalities available for managing personality disorders to present various therapeutic options.
- Implement effective nursing management strategies to ensure comprehensive care for individuals with personality disorders.

Outlines:

- Introduction
- Definition of personality disorders
- Incidence and prevalence
- Risk factors
- Classifications of personality disorders
- Treatment Modalities
- Nursing management

- **Introduction**

Personality disorder is a type of mental disorders in which one have a rigid and unhealthy pattern of thinking, functioning and behaving. A person with a personality disorder has trouble perceiving and relating to situations and people. This causes significant problems and limitations in relationships, social activities, work and school.

Personality disorders usually begin in the teenage years or early adulthood. There are many types of personality disorders. Some types may become less obvious throughout middle age.

- **Definition of personality**

Personality consists of enduring habitual patterns of thinking, feeling, behaving, relating and determine a person's reaction to the two worlds, namely the inner psychological world and the outer environment.

- **Definition of personality disorders**

Personality disorders are long-standing patterns of thought, behavior, and emotions that are maladaptive for the individual or for people around him or her.

- **Incidence:**

Establishing accurate statistics about the incidence and prevalence of personality disorders is challenging because many people with such illnesses do not seek professional help.

In addition, diagnostic criteria for personality disorders have changed frequently, and the professional psychiatric community often questions or calls for re-examination of the different categories and types.

Personality disorders affect 10-15% of the adult US population. The prevalence of any personality disorder was 9.1% in the general population.

- **Risk factors: -**

Although the precise cause of personality disorders is unknown, certain factors seem to increase the risk of developing or triggering personality disorders, including:

- Family history of personality disorders or other mental illness.
- Low level of education and lower social and economic status.
- Verbal, physical or sexual abuse during childhood.
- Neglect or unstable or chaotic family life during childhood.
- Being diagnosed with childhood conduct disorder.
- Variations in brain chemistry and structure.

- **Classifications of personality disorders**

Now the personality disorders classified into clusters or categories based on the predominant or identifying features such as odd and eccentric, dramatic and emotional, anxiety and fear which include the following: -

Cluster A: Includes people whose behavior appears and odd or eccentric

- Paranoid personality disorder.
- Schizoid personality disorder.
- Schizotypal personality disorder.

Cluster B: Include people who appears dramatic, emotional or erratic

- Antisocial personality disorder.
- Borderline personality disorder.
- Histrionic personality disorder.
- Narcissistic personality disorder.

Cluster C: Include who appears anxious or fearful

- Avoidant personality disorder.
- Dependent personality disorder.
- Obsession Compulsive personality disorder.

➢ Cluster A personality disorder

1- Paranoid personality disorder:

It is a pervasive distrust and suspiciousness of others such that their motives are interpreted as malevolent. Beginning by early adulthood.

Clinical manifestations:

- Hypervigilant
- Appear tense and irritable.
- Insensitive to the feelings of others.
- Avoid interactions with other people.
- Some appear to be highly inheritable characteristics.

2- Schizoid personality disorder

It is characterized primarily by a profound defect in the ability to form personal relationship or to respond to others.

Clinical manifestation

- Person with schizoid personality disorder appear cold, aloof and indifferent to others.
- Prefer to work in isolation
- Their behavior and conversation exhibit little or no spontaneity

3- Schizotypal personality disorder

The person behavior patterns characterized by inability to form close relationships and a pattern of cognitive and perceptual distortions and eccentricities

Clinical manifestation

- The persons with this disorder are aloof and isolated and behave in apathetic manner.
- Magical thinking, ideas of reference, illusions, and depersonalization
- The speech patterns is sometimes bizarre
- Person with this disorder often can't orient their thoughts logically and become lost in personal irrelevance
- If these individuals are under stress, they may decompensate psychotic symptoms such as delusion thoughts, hallucinations, or bizarre behavior but they are usually for short duration.

➢ Cluster B personality disorder

1- Antisocial personality disorder.

It is a pattern of socially irresponsible exploitive and guiltless behavior reflects a disregard for the rights of others.

Signs and symptoms: -

- Violation of the rights of others.
- Lying.
- Poor judgment.
- Lack of insight.
- Poor work history.

2- Borderline personality disorder.

It is a pattern of intense relationships, with affective instability with other people.

Signs and symptoms:

- Fear of real - lack of insight
- Unstable relationship - impaired judgment
- Unstable self-image.
- Irritability.
- Suicide attempts and acts of self-mutilation are common.

3- Histrionic personality disorder.

Is a pattern of excessive attention, seeking emotion, usually beginning in early adulthood.

Signs and symptoms: -

- No think before acting.
- Threaten suicide to get attention.
- Have difficulty maintaining relationships.
- Being overly dramatic and emotional.

4- Narcissistic personality disorder.

It is a long-term pattern of exaggerated feeling of self-importance and lack of empathy toward other people.

Signs and symptoms:

- Easily rejected, hurt.
- Thinking about one self and talking about one self a lot.
- Needs constant appraise and admiration.
- Exaggerated response to criticism

➢ Cluster C personality disorder

1- Avoidant personality disorder

It is characterized by a pervasive pattern of social inhibition, feeling of inadequacy, and a hypersensitivity to negative evaluation.

Clinical manifestation

- Self-isolation
- Low self esteem
- Avoid work, social, school activities because fear of criticism or rejection
- Feel frequently unwelcome in social situations.
- Avoid interaction with others
- Feeling nervousness and inadequately
- Sensitivity to criticism or rejection
- Cause is unknown but genetics and environment factors
- Rejection by parents or peers
- Isolation
- Children who experience abuse

2- Obsessive compulsive disorder

Obsessive compulsive personality disorder characterized by excessive concern with orderliness, attention to details and self-control.

Signs & symptoms:

- pre-occupation with orderliness, Rules & schedules
- Fear of being contaminated by germs or contaminating other.
- Fear of losing control.
- Extreme perfection
- Excessive focus on religious or moral ideas.
- Fear of losing or not having things you might need.

3- **Dependent personality disorder**.

Dependent personality disorder characterized by an inability to be alone.

Clinical manifestation:

- Needed repeated reassurance.
- Fear of rejection.
- Feeling isolated and nervous when alone.
- Fearing abandonment.
- Being unable to be alone.
- Being overly sensitivity to criticism.

Nursing management of personality disorder

Hospitalization:

Indication:

- In some cases, a personality disorder may be so severe that require psychiatric hospitalization.
- most inpatient hospitalizations occur due to concern that the patient may be at risk of harming herself or someone
- to keep the person safe during a mental health crisis

Pharmacotherapy:

- Mood stabilizers
- Anti-psychotic reduces symptoms of psychosis and anxiety
- Anti depression to treat depression and hopelessness
- Anti-Anxiety to treat anxiety agitation or insomnia

Psychotherapy

Psychoanalytical

- Therapist tries to take patient into past to know when problem began. It focuses on cause of problem.
- The main goal of this therapy is to bring all represented material to conscious awareness. So, patient can work towards a healthy resolution of problem.

Supportive therapy

In this therapist help patient to relieve emotional distress and change personality by using these techniques

* Ventilation -free expression of feeling or emotion patient encouraged to talk clearly

* Environmental modification through improving well-being of mental patients by change their living conditions

* Reeducate patient regarding his problem and ways of coping.

Behavior therapy

Behavior therapy involved identifying mal adaptive behavior and seeking to correct it

Objective of behavior therapy removing or modifying existing symptoms and promote positive personality growth and development

This therapy most common used in obsessive compulsive personality disorders

Cognitive therapy

Cognitive therapy is psycho therapeutic approach-based idea that behavior is secondary to thinking

It focuses on how patients think about themselves and world

This makes change in current way of thinking, most common applied in

Personality disorders

Individual psychotherapy:

It is a method of bringing about change in a person by exploring his or her feeling, attitudes, thinking and behavior.

Therapy is conduct on a one-to-one basis; the Therapist treats one patient at a time.

Patient generally seek this kind of therapy based on their desire such therapy helps to achieve the following:

_ Understand themselves and their behavior

_ Make personal changes

_ Improve interpersonal relationships

_ Get relief from emotional pain or unhappiness

Group therapy

Group psychotherapy is a treatment in which carefully selected people who emotional ill meet in group guided by therapist and help one another effect personality changes

Selection

* Adolescent and patients with personality disorders

* Families and couples where system needs change

Group size (8-10) members

Each session last for 45minutes to 1 hour

- **Nursing intervention:**

➢ **Nursing interventions cluster A**

- Establish and maintain therapeutic nurse patient relationship.
- Assess client's neurological status to determine if disorder has progressed to another serious conditions such as schizophrenia
- Monitor behaviors and interactions with staff or another client.

- Talk openly with client about their beliefs and thoughts, showing empathy and support to build trust and rapport with client.
- Explain all procedures clearly and carefully with their purpose, before startling them to prevent aggressive behavior and suspicious.
- Discuss feeling and help client identify behaviors that cause conflict or alienate others.
- Minimize environmental stimuli because overstimulation form loud noises, excessive talking, television or radio may increase paranoia and prompt erratic or aggressive behaviors.
- Encourage socialization with others but don't force participation in activities.
- Help reorient client to reality.
- Administer medications appropriately and monitor for reactions to medications.
- Provide education, resources and support for client's family and loved ones.
- Help family members understand the nature of the client's illness and avoid conflict that could exacerbate the client's symptoms.

Nursing intervention for cluster B

- Establish and maintain therapeutic nurse patient relationship.
- Encourage client to examine these behaviors to recognize that they are occurring.
- Give positive reinforcement for independent behaviors.
- Assist client to work toward achievement of object constancy.
- Observe client's behavior frequently during routine activities and interactions.
- Remove all dangerous objects from client's environment.
- Maintain low level of stimuli in client's environment (low lighting, few people).
- Help client to gain insight in to his or her own behaviors.

Nursing Interventions for Cluster C

- Establish and maintain therapeutic nurse patient relationship.
- Assist in setting small, achievable goals.
- Discuss fears and feelings prior to meeting goal.
- Assist to explore feelings.
- Assist to try new activities.
- Assist to decrease anxiety and need for perfection.
- Identify the abilities and strengths and motivate them to utilize it in overcoming or handling stressful situations.
- Encourage them to solve their problems on their own efforts and lead an independent life.
- Motivate the family members to provide situational support and concern.
- Encourage patients to maintain "self-control" technique to overcome ritualistic behaviour.